IS-698 Homework 1 – UB01976

Task 1: Elicitation Augmentation

The Requirement Dataset provided to me is related to a European project called "**ALFRED**" (Adaptable Ambient Living Assistant) which is a voice-controlled personal assistant designed to support elderly individuals who are living independently. It is basically a combination of wearable and smartphone devices which address the problems of old age people like being isolated, their declining health, and becoming dependent on life.

Strategy to Design the Prompt for Task 1 – To design the prompt for task 1 of catching the gaps and ambiguities in requirements documentation and creating a list of questions that must be prioritized and clarified with stakeholders, I have asked the AI model to take up the role of requirements engineer and understand the context first before giving the response. I have provided the AI with the specific format on how the output must be structured so that it can avoid generalizing the answers and gives the response specific to the domain.

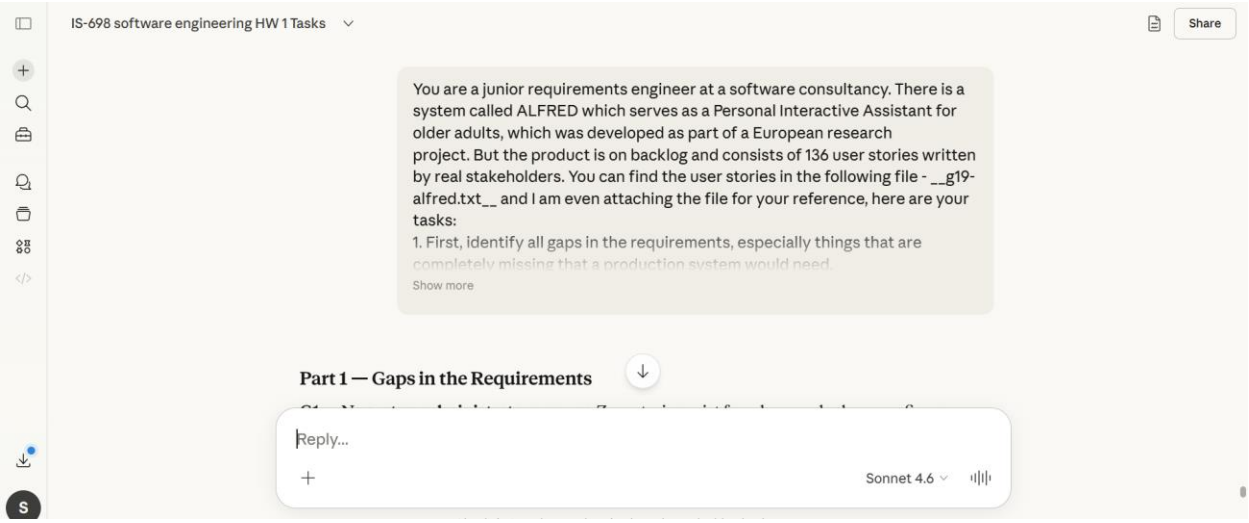
Exact Prompt being Used – Below is the exact prompt used for the model, here for this scenario I have used Claude (Sonnet 4.6) as the AI engine;

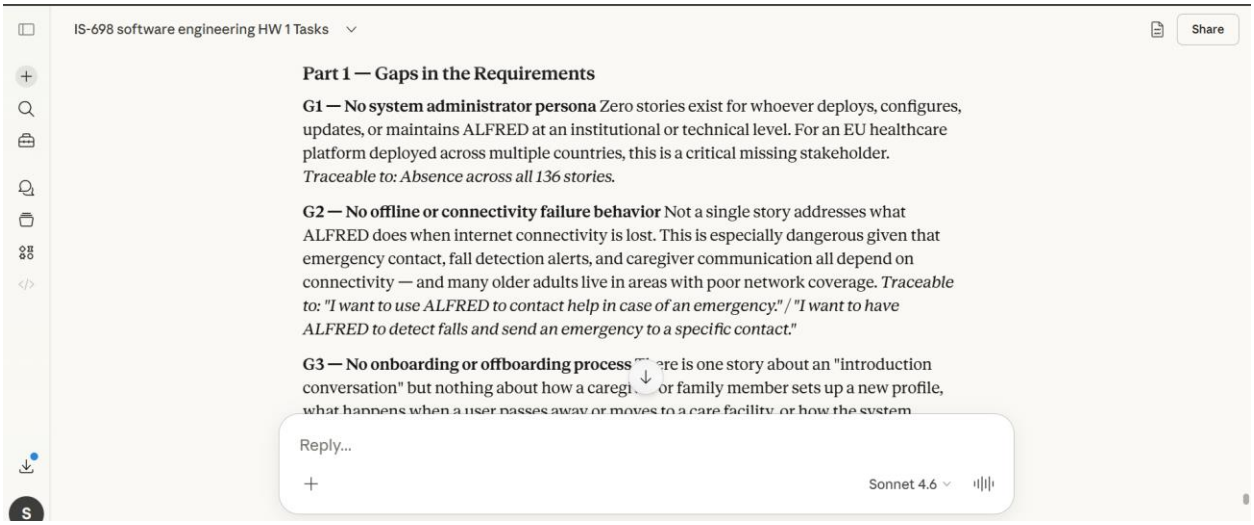
Prompt: "You are a junior requirements engineer at a software consultancy. There is a system called ALFRED which serves as a Personal Interactive Assistant for older adults, which was developed as part of a European research project. But the product is on backlog and consists of 136 user stories written by real stakeholders. You can find the user stories in the following file, g19-alfred.txt and I am even attaching the file for your reference, here are your tasks:

1. First, identify all gaps in the requirements, especially things that are completely missing that a production system would need.
2. Next, identify all ambiguities possible, stories that are too vague to implement without further clarification and missing details.
3. Then, generate a prioritized list of clarifying questions that I can take back to stakeholders if clarification is required. Organize the questions with stakeholder roles as: OlderPerson, MedicalCaregiver, SocialCaregiver, InformalCaregiver, Developer, and System-level. Label each question with the gap or ambiguity it addresses.

Be specific in response. Do not generate generic requirement questions and try not to hallucinate. Every question must be traceable to at least one user's story in the backlog.”

Raw AI Generated Response for TASK 1 – Please find below the attached Screenshots of the AI response to the prompt 1;





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Part 2 — Ambiguities in the Requirements

A1 — "As much as possible with speech interaction" *Traceable to: "I want to use ALFRED as much as possible with speech interaction."* "As much as possible" is not implementable. There is no definition of which features must be voice-accessible versus touch-only, nor any fallback hierarchy when speech fails.

A2 — "Speaks very slowly and clearly" *Traceable to: "I want to have an ALFRED device that speaks very slowly and clearly."* No words-per-minute target, no pronunciation clarity standard, and no specification of whether speech speed is user-adjustable or fixed at one rate.

A3 — "Only reacts to me" contradicts "push a specific button" *Traceable to: "I want to have an ALFRED device that only reacts to me." / "I want to activate the vocal interaction mode of ALFRED by pushing a specific button."* The first story implies passive voice recognition or biometric speaker identification. The second implies manual button

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Part 3 — Clarifying Questions by Stakeholder, Ordered by Priority

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OlderPerson

[A8] Q1. When ALFRED detects a fall, walk me through exactly what should happen next — who gets contacted, how (phone call, text message, or app alert), in what order, and what happens if that person does not answer? *Traceable to: "I want to have ALFRED to detect falls and send an emergency to a specific contact."*

[A11] Q2. You mentioned wanting data stored directly on your phone, but also wanting ALFRED on all your devices. If you update your agenda on a tablet, should your phone automatically reflect that change — or should each device stay independent? *Traceable to: "I want to have my data stored directly on the phone." / "I want to have ALFRED on all my computers, tablets and phone."*

[A3] Q3. You said ALFRED should only react to you — how should it recognize that it is you

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MedicalCaregiver

[A12] Q11. When you want to find where an ALFRED user is — do you need their exact location in real time, or would knowing where they were last seen be enough? Would you expect the user to agree to this once upfront, or each time you check? *Traceable to: "As a MedicalCaregiver, I want to locate the ALFRED user."*

[G5] Q12. What rules or regulations does your organization follow when accessing patient data? Are there any existing health record systems that ALFRED would need to connect with? *Traceable to: "I want to know exactly what ALFRED does with my personal data, and share it only on my specific permission."*

[A13] Q13. The backlog includes a story about ALFRED giving tips to reduce health impairments without medication — who should be responsible for making sure those tips are medically accurate? Should a doctor or nurse review them before they reach the user? *Traceable to: "I want to receive specific tips how to reduce my health impairments without*

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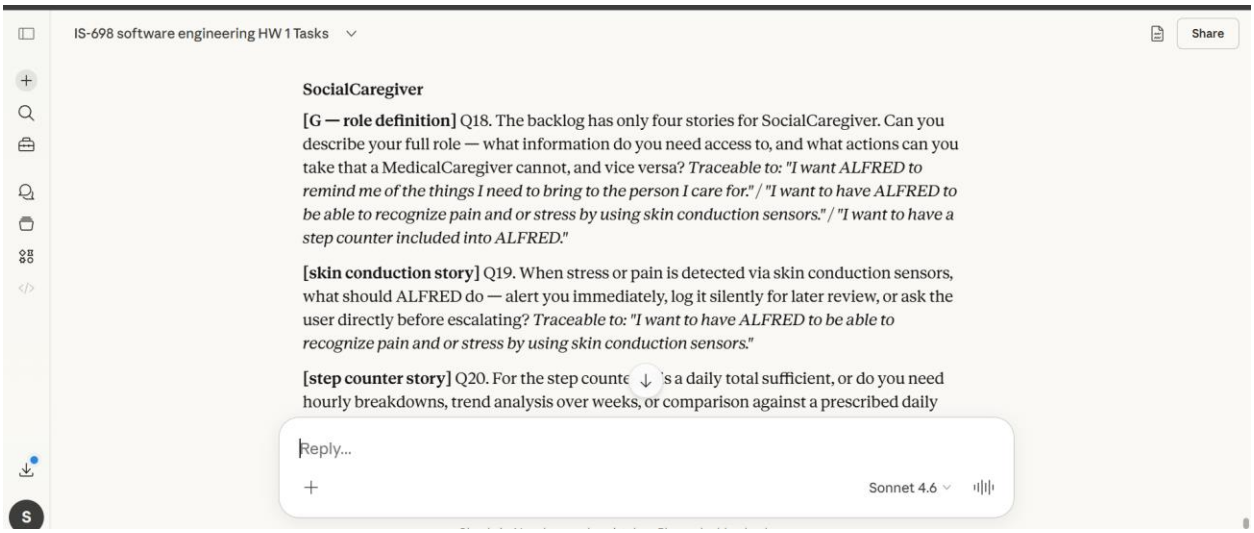

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SocialCaregiver

[G — role definition] Q18. The backlog has only four stories for SocialCaregiver. Can you describe your full role — what information do you need access to, and what actions can you take that a MedicalCaregiver cannot, and vice versa? *Traceable to: "I want ALFRED to remind me of the things I need to bring to the person I care for." / "I want to have ALFRED to be able to recognize pain and or stress by using skin conduction sensors." / "I want to have a step counter included into ALFRED."*

[skin conduction story] Q19. When stress or pain is detected via skin conduction sensors, what should ALFRED do — alert you immediately, log it silently for later review, or ask the user directly before escalating? *Traceable to: "I want to have ALFRED to be able to recognize pain and or stress by using skin conduction sensors."*

[step counter story] Q20. For the step counte s a daily total sufficient, or do you need hourly breakdowns, trend analysis over weeks, or comparison against a prescribed daily

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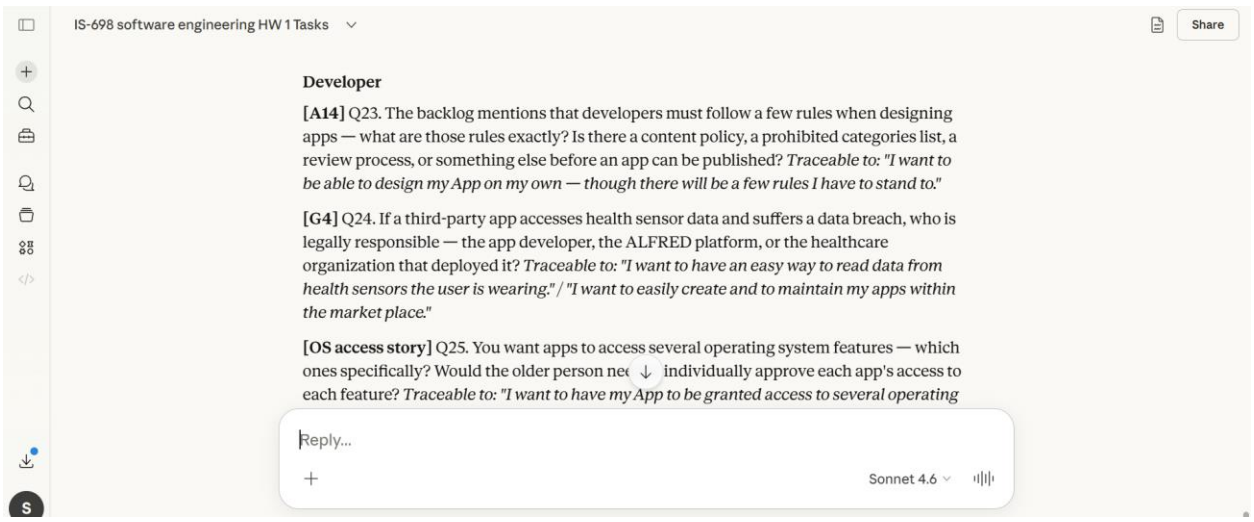

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Developer

[A14] Q23. The backlog mentions that developers must follow a few rules when designing apps — what are those rules exactly? Is there a content policy, a prohibited categories list, a review process, or something else before an app can be published? *Traceable to: "I want to be able to design my App on my own — though there will be a few rules I have to stand to."*

[G4] Q24. If a third-party app accesses health sensor data and suffers a data breach, who is legally responsible — the app developer, the ALFRED platform, or the healthcare organization that deployed it? *Traceable to: "I want to have an easy way to read data from health sensors the user is wearing." / "I want to easily create and to maintain my apps within the market place."*

[OS access story] Q25. You want apps to access several operating system features — which ones specifically? Would the older person nee individually approve each app's access to each feature? *Traceable to: "I want to have my App to be granted access to several operating*

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Please find the attached link to the document consisting of the complete AI response: -

https://github.com/UB01976/Sotware_Engineering_698/blob/main/Homework1_Raw_AI_Response.pdf

Critical Evaluation of the Output -

The AI model successfully identified most of the gaps and ambiguities in the requirements. It was able to clearly identify most of the contradictions related to the architecture of the overall application, such as “data stored directly on phone” and at the same time “ALFRED on all computers, tablets, and phones" this requirement is a clear ambiguity as it directly contradicts the design because it requires both the storage of data locally as well as multi-device sync. This might not be understood clearly by a junior level requirements engineer at the very beginning itself. It was also able to identify most of the emergency escalation gaps, i.e. in most of the user stories there were many details missing such as whom to contact in emergency? How should the alert be sent? And

identified all these types of gaps where the required information was missing and detected them across multiple stories.

The AI even identified the duplicate stories at the end of dataset as well, which is a possible data quality issue, and this sort of issues can be sometimes missed by under experienced reviewers as they accept them because those duplicate stories are syntactically still valid. Another important gap which was primarily highlighted was who was in authority or who handles the ALFRED operations. In the complete dataset, there is no single user story about who is going to configure and maintain the ALFRED at the organizational level. Even the regulatory and compliance requirements gap was correctly noted by the AI. As this app needs approval from health administrations, compliance plays a major role, and no proper approval can lead to blocking of the whole application deployment. All the questions were even properly categorized into groups of which one is to be asked whom – like whether to the Medical Caregiver, or Developer or the Older person.

However, there are few cases which I have found that were **overlooked by AI and were not caught by it**, such as the duplicate requirements and grammatically malformed ones. Humans would have caught them immediately as soon being noticed, whereas AI considered them still as valid ones and did not flag anything. One more important thing is that AI did not consider the place where the requirements are made from and consortium partners involved; it simply treated everything equally which might not be possible for a project. There were even requirements with typo errors (Like AFLRED instead of ALFRED), and it did not specify how the care givers want to receive the notifications, which shows that LLM did not consider them at all.

There were however some **Leading** Questions such as “if ALFRED needs to speak slowly, how slow is actually slow and is there any speed fixed for it”? Another important leading question is “if the internet connectivity is lost or the device goes offline, how should it contact an emergency is it through direct cellular or wait for connection to be restored”. As these were never mentioned so they lead to new ways on how to address them. There were even a few **inappropriate or low-quality questions** related to whether ALFRED complies with the EU standards or not, and the format necessary for the daily feedback from ALFRED or is a weekly report required.

When it comes to the evaluation metrics, for TASK 1, the AI model produced **Correctness of 88 %,** out of 35 questions produced in total 30 were correctly traceable to the actual user stories in the dataset. 5 were leading or introduced new questions which were exactly not related to the dataset.

Precision is 87 % most of the questions handle the user stories, **Completeness is 85 %** as some duplicates were identified and missed the ones with the grammatical errors. The **Relevance stands at 90%** as most were relevant, but a few were inappropriate to the stakeholders which involved the technical regulations of EU medical standards and **bias or divergence is low**.

Improved Human-Validated requirements -

The requriements which were generated by AI are accurate and cover all most all the user stories from the dataset. But as mentioned earlier in the critical evaluation, the AI missed few such as the duplicates, Typos, malformed, and overlapping stories related to visual and hearing impairment. So, these were a few of the additional requirements which I have found during the analysis of the dataset and are added to the requirements list. There were even no requirements covered related to the notifications from the AI, so they are added later after human evaluation. Please find below link for the improved human validated requirements document with additional requirements -

https://github.com/UB01976/Sotware_Engineering_698/blob/main/Homework1_Improved_Human_Validated_Requirements.pdf

Task 2: Requirements Extraction and Classification

Strategy to Design the Prompt for Task 2 – For task 2 as the AI has already assumed the role of requirements engineer and has understood the ALFRED dataset. I have structured the prompt to produce the response consisting of users stores properly extracted into requirements and classified into respective subcategories. Even made sure that it does not hallucinate and produce new requirements, and trace back to the actual user stories in the dataset.

The AI prompt for extracting and classifying the requirements from the dataset is as follows;

Prompt - "For the same dataset g19-alfred.txt could you classify and extract requirements from it into the following four categories: Functional Requirements (FR), Non-Functional Requirements (NFR) with subcategories such as usability, security, performance, portability, and reliability, Business Rules, and Constraints. Be specific and do not assume. Every requirement you extract must be traceable to user story in the backlog. Do not invent requirements that are not present in the stories."

Raw AI Generated Response for TASK 2 – Please find below the attached Screenshots of the AI response to the prompt 2;

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For the same dataset g19-alfred.txt could you classify and extract requirements from it into the following four categories: Functional Requirements (FR), Non-Functional Requirements (NFR) with subcategories such as usability, security, performance, portability, and reliability, Business Rules, and Constraints. Be specific and do not assume. Every requirement you extract must be traceable to user story in the backlog. Do not invent requirements that are not present in the stories.

Functional Requirements (FR)

FR1 — Speech & Interaction

FR1.1 The system shall deliver information to the user via speech technology

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Functional Requirements (FR)

FR1 — Speech & Interaction

FR1.1 The system shall deliver information to the user via speech technology.

FR1.2 The system shall support speech as the primary interaction mode.

FR1.3 The system shall allow the user to activate voice interaction mode by pressing a specific physical button.

FR1.4 The system shall repeat the last spoken phrase on user request.

FR1.5 The system shall support speech input for inserting meetings and agenda entries.

FR1.6 The system shall conduct an introductory conversation with the user during first setup to learn user preferences.

FR1.7 The system shall only listen to the user when explicitly asked to do so.

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Non-Functional Requirements (NFR)

NFR1 — Usability

NFR1.1 The system shall display only well-visible large buttons suitable for older adults.

NFR1.2 The system shall provide visual support when displaying lists, maps, and agendas.

NFR1.3 The system shall allow the user to zoom into all visual content.

NFR1.4 The system shall speak at a slow and clear pace appropriate for older adults.

NFR1.5 The system shall be adaptable for users with visual impairments.

NFR1.6 The system shall be adaptable for users with hearing impairments.

NFR1.7 The system shall allow the user to cu ize preferences across all functionalities.

NFR1.8 The system shall come fully preinstalled and configured so the user does not need

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NFR2 — Security & Privacy

NFR2.1 The system shall ensure that personal data is shared only with the user's explicit permission.

NFR2.2 The system shall ensure the MedicalCaregiver is confident that the user retains control over who accesses their data.

NFR2.3 The system shall store user data directly on the user's phone.

NFR3 — Performance

NFR3.1 The system shall maintain sufficient battery life to operate throughout the entire day without recharging.

NFR3.2 The system shall deliver developer app status notifications in near real-time.

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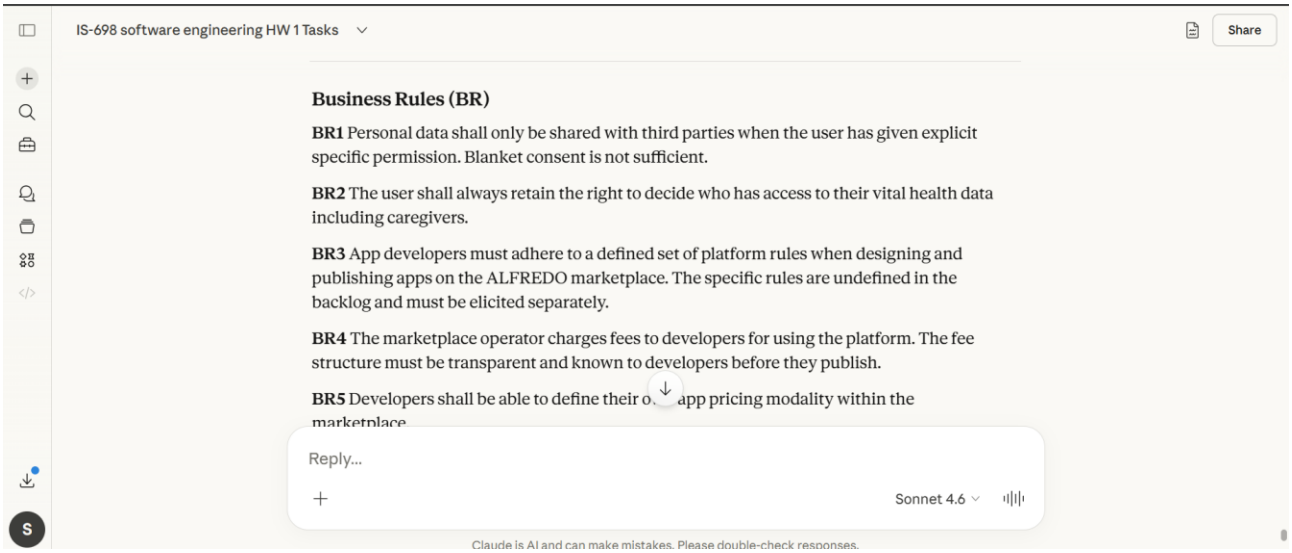

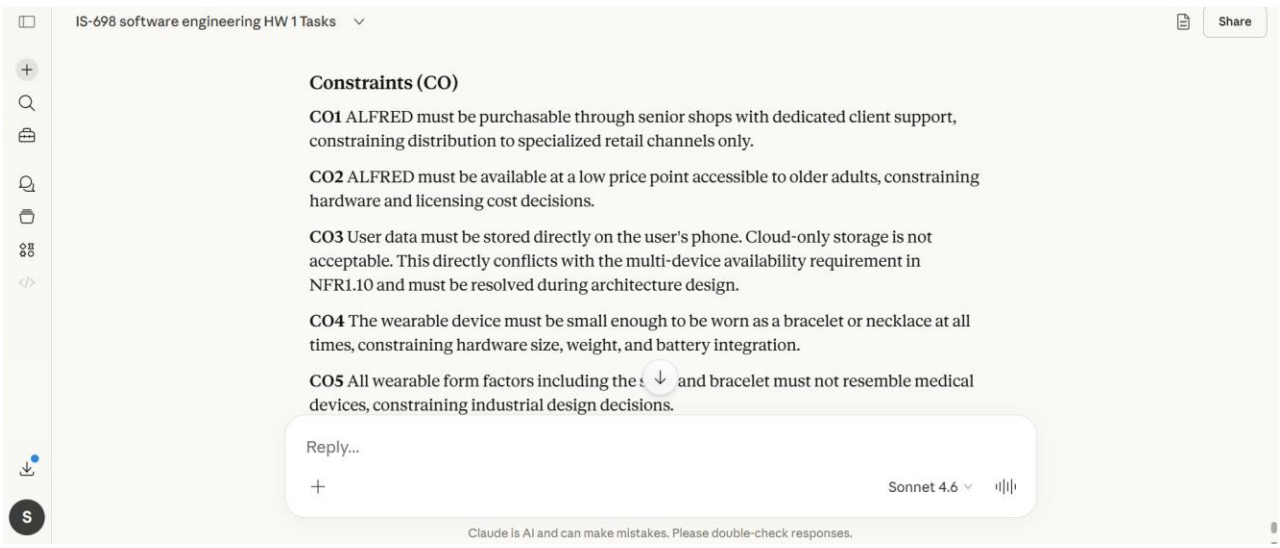

Please find the attached link to the document consisting of the complete AI response: -

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Evaluation of requirements extracted and classified – The model has accurately produced the requirements from the dataset provided. It was also able to classify most of them in the categories of functional, non-functional, and business rules. It had handled all the classifications and duplicates as well without any issues, and the overall coverage was also very well. However, there were few issues like misclassifying some as functional instead of business as they did not have any input, output, or measurable outcomes. It also did not account for the game developer and caregiver; these categories did not get their respective categories in functional requirements though they had a distinct role. Overall, the AI model was able to classify and extract most of the requirements from the dataset, apart from the duplications between non-functional and constraints,

misclassifying one as functional, and assumption in one most of the others were fine. These are not serious issues which might cause the roadblocks in building in the project, but a senior requirements engineer might have caught them and flag them as issues.

The evaluation metrics of task 2 are as follows; **Correctness is 90 %** there we few errors such few which must be constraints classified as non-functional requirements (related to aesthetics), few business rules misclassified as functional, and some assumptions is making functional requirements such as system supporting the body weight of user. The **precision is at 92%** and the **Completeness at 88%.** There is a slight dip in completeness because it did not group the gamers separately, and app personalization aspect was completely ignored.

Improved Human-Validated requirements – While AI was able to classify all the requirements into the functional, non-functional, business rules, and constraints. There were few improvements required related to scope and clarification from the clients as well; payment security and fraud detection were not covered from the dataset also. There is even an issue with the local storage, which was missed by AI, and added after the human validation. There are constraints that are to be defined after the marketplace is defined, so they are added once after proper human evaluation. Few of the non-functional requirements were also removed, which were replicated similarly in the constraints. Please find the link to the human evaluated requirements document -

https://github.com/UB01976/Sotware_Engineering_698/blob/main/Homework1_Improved_Human_Validated_Requirements.pdf

Task 3: Conflict Detection and Prioritization

Strategy to Design the Prompt for Task 3 – For designing the prompt for task 3 we basically require the AI model to assume the role towards judgement side rather than extracting, this way it can concentrate better on reasoning and understand the consequences of implementing those requirements. Next thing is prompt must reference the output from the above task 2, as we need to process the requirements generated not the raw dataset. Because AI should not return back to the initial original dataset as that would not align with the task 3 needs.

The other most important thing which must be properly addressed in the prompt is that conflict detection and Moscow prioritization must be clearly separated into two separate instructions because if not they get mixed up into one single set and it's difficult to analyze which are conflicts and which are related to prioritization. The AI model is asked to provide the prioritization order on what must be, can be, and may not be. Because without this

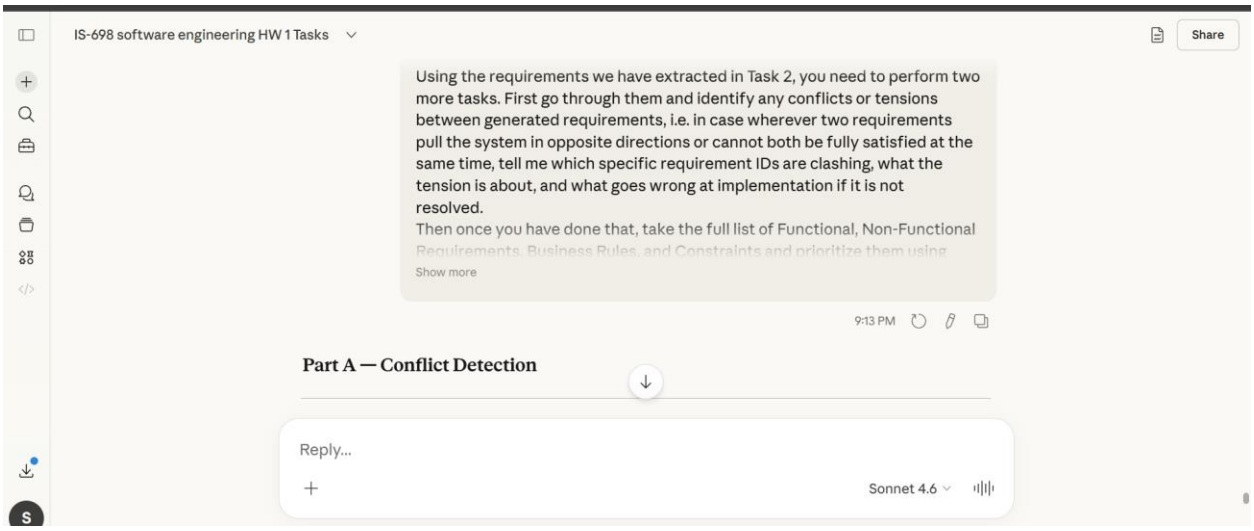
the model weighs all the requirements equally and will not prioritize the safety ones over the entertainment ones.

The AI prompt used for the conflict detection and prioritization is as follows;

Prompt - "Using the requirements we have extracted in Task 2, you need to perform two more tasks. First go through them and identify any conflicts or tensions between generated requirements, i.e. in case wherever two requirements pull the system in opposite directions or cannot both be fully satisfied at the same time, tell me which specific requirement IDs are clashing, what the tension is about, and what goes wrong at implementation if it is not resolved.

Then once you have done that, take the full list of Functional, Non-Functional Requirements, Business Rules, and Constraints and prioritize them using MOSCOW criteria, like which of them are "Must Have", "Should Have", "Could Have", and "Won't Have". Also provide a short justification for each, keeping in mind that the anchor for what counts as Must Have is whatever is essential for a safe, legally compliant, and working minimum viable product for older adults in a European healthcare context. Do not invent any new conflicts or priorities, everything must be directly traceable to the requirements list created in task 2.**"**

Raw AI Generated Response for TASK 3 – Please find below the attached Screenshots of the AI response to the prompt 3;



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Part A — Conflict Detection

Conflict 1 — Local data storage vs. multi-device availability Requirements: NFR2.3 / CO3 vs. NFR1.10

NFR2.3 and CO3 both require user data to be stored directly on the phone. NFR1.10 requires ALFRED to be available across all of the user's computers, tablets, and phones. These are architecturally incompatible. If data lives only on the phone, a tablet cannot access it without network sync. If data syncs across devices, it cannot be stored locally only. If left unresolved, the development team cannot make a fundamental architecture decision — any choice they make will violate one of the two requirements.

Conflict 2 — User privacy and data control vs. caregiver monitoring Requirements:

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Conflict 3 — Autonomous fall detection vs. user control over ALFRED Requirements: FR4.1, FR4.3 vs. FR12.5

FR4.1 requires ALFRED to autonomously detect falls and send emergency alerts without user instruction. FR12.5 requires the user to always retain full control over all ALFRED activities. Autonomous fall detection by definition acts without the user triggering it — it fires automatically. If taken literally FR12.5 would prevent autonomous emergency responses, making the safety features non-functional. If left unresolved, either the safety feature is disabled to preserve user control, or user control is silently overridden to preserve safety.

Conflict 4 — Proactive questioning vs. user control and alert fatigue Requirements: FR5.7 vs. FR12.5, NFR1.7

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Conflict 9 — Encouraging face-to-face contact vs. comprehensive digital social services Requirements: FR6.9, BR8 vs. FR6.1, FR6.2, FR6.3, FR6.6, FR6.7, FR6.8, FR7.3, FR11.6

The system is required to encourage real human interaction and not position itself as a replacement for human contact. At the same time it provides video calls, messaging, social group formation, event invitations, and online multiplayer games. The more capable and convenient the digital social features become, the more they risk becoming a substitute for the face-to-face contact the system is supposed to encourage. If left unresolved no developer can determine when a social feature crosses the line from supportive to substitutive.

Conflict 10 — Daily exercise variety vs. MedicalCaregiver safety oversight Requirements: FR11.13 vs. FR11.16, FR11.19

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Part B — MoSCoW Prioritization

Must Have

Essential for safety, legal compliance, and basic system function on day one.

FR3.1 — Medicine reminders to the user. *A missed medication dose for an older adult can be life-threatening. Non-negotiable.*

FR3.2 — User medication self-check to avoid overdose. *Directly prevents patient harm. Must be present at launch.*

FR3.3 — MedicalCaregiver medication adherence check. *Clinical oversight of medication compliance is a core healthcare requirement.*

FR4.1 — Fall detection and emergency alert. *Falls are the leading cause of injury in older adults. Core safety feature.*

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Should Have

Important for usability and user wellbeing but not immediately safety-critical.

FR1.3 — Activate voice mode via physical button. *Useful fallback but not the only interaction mechanism.*

FR1.4 — Repeat last spoken phrase. *Important accessibility feature but system functions without it.*

FR1.5 — Speech input for agenda entries. *Enhances usability but agenda can be managed without speech input.*

FR1.6 — Introductory setup conversation. *Improves onboarding experience but not safety-critical.*

FR1.9 — Developer-defined user questions via API. *Important for platform extensibility but not MVP-critical.*

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Could Have

Valuable features that enhance experience but not essential for MVP.

FR2.7 — Sensor data capture during exercises. **FR2.8** — Pain and stress recognition via skin conduction. **FR5.6** — SocialCaregiver reminder of items to bring. **FR5.7** — Proactive interest-based questions at scheduled times. **FR6.3** — Video conversations. **FR6.4** — MedicalCaregiver videophone. **FR6.6** — Contact people from own social circle. **FR6.7** — Social groups by similar diseases. **FR6.8** — Organize group meetings. **FR6.9** — Encourage face-to-face interaction. **FR7.1** — Art exposition and museum information. **FR7.2** — Neighborhood social activity information. **FR7.3** — Personalized social event invitations. **FR7.4** — Neighborhood sports class suggestions. **FR7.5** — App recommendations matching user interests. **FR8.1** — Navigation assistance. **FR8.2** — Navigation to events. **FR8.3** — Navigation for mild cognitive impairments. **FR8.4** — Public transport support. **FR8.5** — Taxi arrangement support. **FR8.6** — Touristic walk guidance. **FR9.1** — Door

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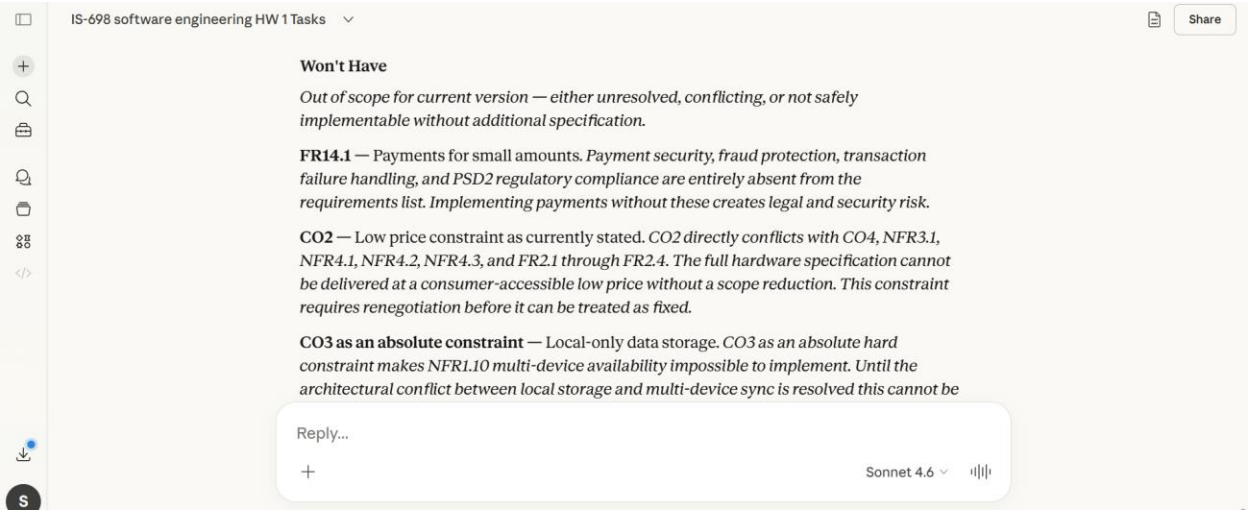

Please find the attached link to the document consisting of the complete AI response: -

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Critical Evaluation of Conflicts discovered and Prioritization of requirements –

The LLM model has evaluated most of the conflicts accurately and was tracing back them correctly to the generated requirements. There was a real issue with conflict 3 which was analytical and it contradicts fall detection with the user control management. AI managed well to identify it and the consequence of implementation, i.e., either the safety fails or the requirement overriding. Conflict 5 (related to the sensor data collection vs informed consent) was also well reasoned in terms of EU healthcare systems. While there were some which were missed by the model, such as conflict between the absence of daily caregiver vs the varying exercises. While conflict 10 partially addresses it, there is no official mechanism to prevent any new harmful exercise from being assigned autonomously.

The MosCow prioritization was also well aligned as any real-world requirements engineer would have prioritized the Must have section with the safety and privacy related requirements. And as it serves older people, the basic accessibility of application is non-negotiable from day one itself. The placement of Conflict 2 (Price constraint) in the won't have section is also technically accurate as any requirements engineer would immediately flag it and raise concerns to client as it's related to budget. However, AI sometimes got diverged when using MosCow prioritization, i.e. pushing the whole developer API into the could have section. While a real practitioner would argue that a platform with minimum requirements is required to be considered viable. Also, some of the important constraints related to navigation for impairments and medical care giver locations are placed in could have, which must have been in the should have section considering the significance of the audience here being old.

Coming to the evaluation metrics, the **Correctness is 88 %** considering few conflicts being missed, **Precision is 85 %** because sometimes the MosCow prioritizations have become generic. The **Completeness stands at 80 %** and **Relevance at 87 %** as it missed few cognitive distress risks and grouping conflicts. The Bias/ Divergence of the output response is **Moderate** as it underweighted the development side and overweighted the generic side.

Improved Human-Validated – While in the AI's response only 10 conflicts were detected, an additional 3 were observed while going through the whole requirements document. Those were related to the sensitive data exposure during the grouping of the diseases, and two others relating to the cognitive impairment and distress caused by the unsolicited questions from the application. Coming to the MosCow analysis, few related to legal constraints which must be in Should have are misclassified into Could have needed correction. There were several features related to the API platform such as third-party integration and extensibility, which required platform but was moved to could have section. These carry out several unresolved conflicts and need to be addressed before the actual implementation of the system, as they are a high priority. Please find the link to the human evaluated requirements document -

https://github.com/UB01976/Sotware_Engineering_698/blob/main/Homework1_Improved_Human_Validated_Requirements.pdf

Conclusion

Overall, a detailed analysis was performed on the dataset related to ALFRED (AI tool healthcare tool) assisting the older groups. Firstly, the dataset was made of raw user stories from various stakeholders involved (like the older people, care givers, medical givers) and involved several use cases from various groups; it was segregated properly. Identified the gaps and ambiguities which are in the provided user stories and made a list of questions that to have to be discussed with the stakeholders for further clarification and information. AI tool (Claude) was used to generate this, by providing it a proper prompt to generate the requirements. Next, the generated requirements were classified into respective categories of Functional, Non-functional, Business rules, and Constraints. Verified if there were any conflicting requirements (such as the local storage vs cloud) and prioritization which have to be done first and what can be carried out later. Once the list is generated, it was validated and necessary improvements were carried out for the ones that required and made the final improved requirements document. These refined requirements help to design AI assistants like ALFRED error free in domains like healthcare and assisting older individuals.